

InPursuit Health is the *infrastructure* for person-centric clinical services.

THE GAP NOBODY HAS CLOSED

Two sides of healthcare have been waiting for the *same thing*.

THE CONSUMER SIDE

330M

Americans can finally control their own healthcare data



THE PROVIDER SIDE

230K

Small and mid-sized providers can now offer digital health services

ENGINE 01 • D2C APP MEMBERSHIP

The InPursuit App.

BETTER OUTCOMES • SAVE TIME • SAVE MONEY

Universal consumer app on Apple App Store, Google Play, and the Medicare App Library. **Free / Plus / Premium** membership tiers (Individual & Family). **Live at the mid-July 2026 White House launch.**

Harness the Power of Your Data™

ENGINE 02 • THE INPURSUIT PARTNER NETWORK

Independent practices, *one infrastructure*.

NEW REVENUE • DIGITAL SERVICES • AI BILLING

InPursuit bills directly under its own NPI across **every payer** — RPM, CCM, TCM, BHI, ACCESS. Referring practices earn **CMS co-management payments** plus new digital-service revenue, with AI surfacing eligible patients and care gaps. No revenue share. **Phase 1 launches four weeks after bridge close.**

Results Matter™

Concierge Layer

Serving the segment that pays for time, access, and continuity the system can't deliver.

ANCHOR PRACTICE SIGNED

Honeycomb Medical Group — Memphis, TN. ~22,000 patients on eClinicalWorks. Our first infrastructure customer — the proof that Engine 2 is real before a dollar of bridge capital is spent.

~\$3.0M
YEAR 3 RUN-RATE

COMBINED YEAR-3 RUN-RATE • ENGINE 1 + ENGINE 2 • CONSERVATIVE ~10K BASIS

5 MEMBERS

\$10M

25 MEMBERS

\$36M

50 MEMBERS

\$68M

100 MEMBERS

\$132M

Honeycomb is our **representative customer** — live proof of the model. The network is modeled conservatively at ~10K panels, half of Honeycomb. Figures are **steady-state run-rate at full ramp; no revenue share**. Each rung = members × ~\$1.28M (Engine 2) + ~\$3.67M (Engine 1, flat).

“We are InPursuit of a more engaged, connected, efficient, proactive, and Person-Centric Healthcare Ecosystem — *where the data follows the patient, the care follows the data, and the payment follows the outcome.*”

THE INVESTMENT THESIS

\$2M bridge. *Phase 1 in four weeks. Series A in Q4.*

The bridge closes on a rolling basis — first money in unlocks Phase 1 within four weeks of close. **Available as SAFE or Convertible Note at a \$20M post-money cap, no discount, \$50K minimum check.** Bridge converts at the Series A at favorable terms. Series A target: **\$13M, Q4 2026 close**, post-launch and post-credentialing. One cap table. Two compounding revenue lines.

NOW • THE BRIDGE

\$2M

SAFE or Convertible Note · \$20M post-money cap
\$50K min check · No discount · Rolling close
Phase 1 launches 4 wks after close

→

Q4 • 2026 • SERIES A

\$13M

Priced round (Series A) · Lead investor TBD
Bridge converts at favorable terms
Funds Phase 2 + sales execution

INSTRUMENT

SAFE or Convertible Note

VALUATION CAP

\$20M post-money

MINIMUM CHECK

\$50K

NEAR-TERM MILESTONES

APR 09 • 2026

CMS First Wave selection

InPursuit named to the **CMS Health IT Ecosystem First Wave** at the April 9 Washington, D.C. launch event. The only participant delivering integrated condition management, paperless intake, and AI-powered health assistance on patient-owned infrastructure.

MID-JULY • 2026

White House launch · Medicare App Library live

App Store + Google Play + Medicare App Library distribution simultaneously. **67M Medicare beneficiaries can discover the app on day one.** Engine 1 goes live.

Q4 • 2026

All-payer direct billing live · Series A close

CMS-855B approved. **Direct billing live across every payer** — Medicare, Medicare Advantage, commercial, Medicaid, cash — for RPM, CCM, TCM, BHI, ACCESS. The **revenue inflection point**. \$13M Series A targets Q4 2026 close.

YEAR 3 • COMBINED RUN-RATE • 25 MEMBERS AT ~10K PANEL

~\$36M_{run-rate}

Engine 1 ~\$3.67M D2C (held flat at Y1 exit ARR) · **Engine 2** ~\$32M Partner Network — 25 conservative members at ~10K panels. Honeycomb, our representative customer, runs higher at its actual ~22K scale.

Upside, excluded from the base: iOS / Android growth, ACCESS OAPs, and CIN participation — all unmodeled.

USE OF PROCEEDS

\$500K PHASE 1 CLINICAL STAND-UP · NPI · CMS-855B · CAQH	\$500K PRODUCTION LAUNCH & PAYER INTEGRATIONS	\$300K HONEYCOMB ONBOARDING · 22K- PATIENT PANEL MIGRATION	\$300K COMPLIANCE & GTM · SOC 2 · HITRUST E1	\$400K OPEX & RESERVE · RUNWAY TO SERIES A CLOSE
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“What \$2M actually buys: ownership in the infrastructure for a *person-centric healthcare world*. And the capital to scale from one Honeycomb to 25 providers on infrastructure designed for unlimited scale.”